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MULTI-CURRENCY HEALTHCARE GROUP | FP&A AND CONSOLIDATION

Ravaan Medical Group: is the cash problem a revenue problem?

A fictional group of 18 clinics in 8 countries, built end to end: realistic monthly actuals, budget and prior year; consolidated statements with proper FX translation; patient visits, doctor productivity and insurance claims; a 13-page Power BI pack with 209 doctors' data and a decision simulator.

AED 300.5m
net revenue Jan-Sep 2026, +14.4% vs last year

13.2%
EBITDA margin (budget 16.0%)

95.8%
of revenue converted to cash

13 / 13
reconciliation checks pass

THE BRIEF

One question per stakeholder

CFO
Did we grow, or did the currencies move? How far are we from budget, and why?

Revenue-cycle lead
Cash is below revenue in Saudi Arabia. Slow insurers, rejected claims or weak sales?

COO
Which branches and doctors earn the profit, and which ones need a decision?

Board
Open a clinic, hire doctors, raise prices or close a branch: what does each do to group EBITDA?

THE GROUP

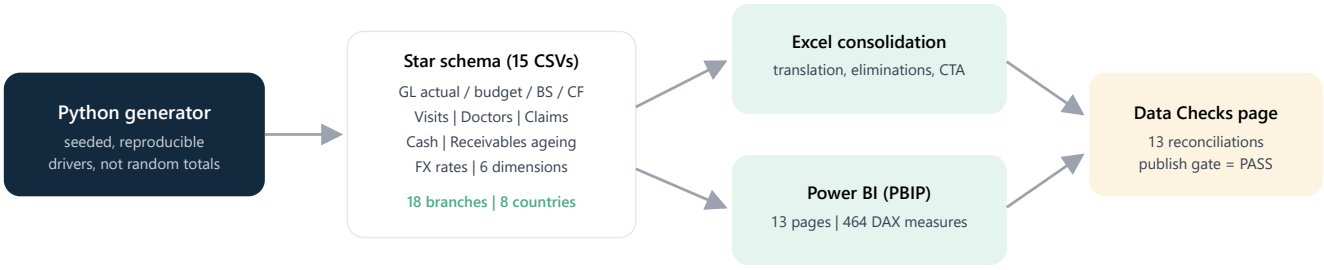
18 clinics, 8 currencies, one reporting currency

Ravaan Medical Group Holding (Dubai) owns eight operating companies: UAE (4 clinics), Saudi Arabia (3), Qatar (2), Kuwait (2), Pakistan (2), Afghanistan (1), UK (2) and USA (2). SAR, QAR and USD are pegged to the dollar like the AED; KWD follows a basket; **GBP, PKR and AFN float** - which is where translation hurts. Eight departments per clinic, 26 payers (self-pay, insurers, government and NGO programmes) and 209 named doctors.

Design choice	Why it matters
Revenue earned, claim submitted, approved/rejected and cash received are four separate events	The only way to tell slow payment from rejected claims from genuine revenue weakness
P&L at monthly average rates, balance sheet at closing rates, equity at historical rates	IAS 21 translation; the difference lands in the translation reserve, not in profit
Every revenue AED is visits x tariff - discount	The GL, the visit table and the doctor table reconcile to the cent
Budget at a fixed budget rate; prior year at its own rates	Separates operational variance from FX variance

HOW IT IS BUILT

One seeded generator, three outputs that agree



The data is generated from operating drivers - doctors, slots, utilisation, tariffs, payer mix, contract discounts, rejection rates, payment lags, rent, salaries and FX - so every total is the result of a business mechanism, and the story can be traced to its cause.

FEATURE 1

Multi-currency consolidation: growth vs FX

Reported revenue grew **+14.4%**. At last year's exchange rates it would have grown **+15.5%**: FX took **AED 3.0m** off revenue and AED 0.8m off EBITDA. The group view hides how uneven that is:

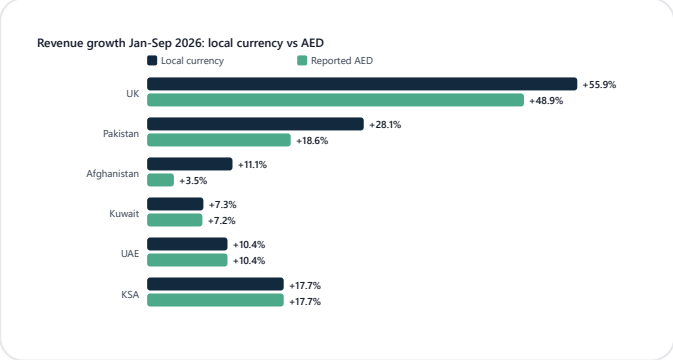
Pakistan +28.1% in rupees, only +18.6% in AED.

UK +55.9% in sterling (new Manchester clinic), +48.9% in AED - the largest FX drag (AED 2.4m).

Afghanistan +11.1% locally, +3.5% reported.

GCC + USA pegged currencies: local and AED growth are identical.

On the balance sheet the weaker rupee and pound produced a translation reserve of **AED -6.5m** at 30 September 2026 (Pakistan AED -5.3m, UK AED -0.9m) - booked in equity, built in the workbook with live formulas.



FEATURE 2

Insurance revenue cycle: why cash trails revenue

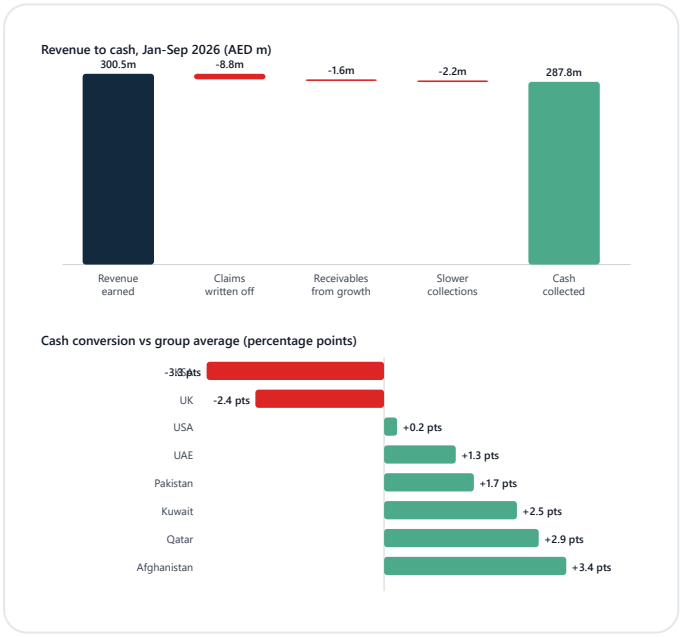
The group converted **95.8%** of revenue into cash. The cash bridge splits the gap into three causes - each needs a different owner:

Rejected claims AED 8.8m written off after resubmission.

Growth AED 1.6m more receivables simply because revenue is higher - healthy.

Slow payment AED 2.2m held back because insurers paid slower (DSO up).

Saudi Arabia is the outlier: revenue +17.7% but cash conversion only 92.5%. One insurer, Najd Care, raised its initial rejection rate from 6.4% in Jan-25 to 13.9% in Sep-26 and pays later - KSA DSO moved from 93 to 103 days. Diagnosis: **not a revenue problem** - a payer-contract and coding problem.



FEATURE 3

Branch investment simulator

A what-if page runs on the last 12 months of actuals (revenue AED 395.6m, EBITDA AED 53.9m). Each lever is a disconnected slicer; the bridge shows which decision moves EBITDA.

Decision tested	EBITDA effect	Read
Raise prices 3% (elasticity -0.3)	+AED 8.9m	Largest single lever - but needs payer negotiation
Cut claim rejections by 3 points	+AED 5.1m	Fixes the KSA problem; also lifts cash
Collect insurers 10 days faster	cash +AED 8.0m	One-off cash release, no profit effect
Close Chicago River North	+AED 0.9m	Removes losses; one-off closure cost shown separately
Close Al Wakrah (Qatar)	+AED 0.6m	Utilisation problem - fix before closing
New polyclinic - UAE (Dubai South)	AED -2.7m yr 1	Mature EBITDA AED 3.6m, payback 2.8 years
New polyclinic - KSA (Riyadh East)	AED -2.6m yr 1	Mature EBITDA AED 4.1m, payback 2.4 years
New clinic - UK (Birmingham)	AED -4.4m yr 1	Mature EBITDA AED 6.8m, payback 2.1 years
New clinic - Pakistan (Islamabad)	AED -0.5m yr 1	Mature EBITDA AED 0.8m, payback 3.7 years

FINDINGS

What management should do

EBITDA is **AED 39.6m**, AED -10.7m below budget. The bridge shows it is a **revenue shortfall** (AED -14.7m), partly offset by lower costs:

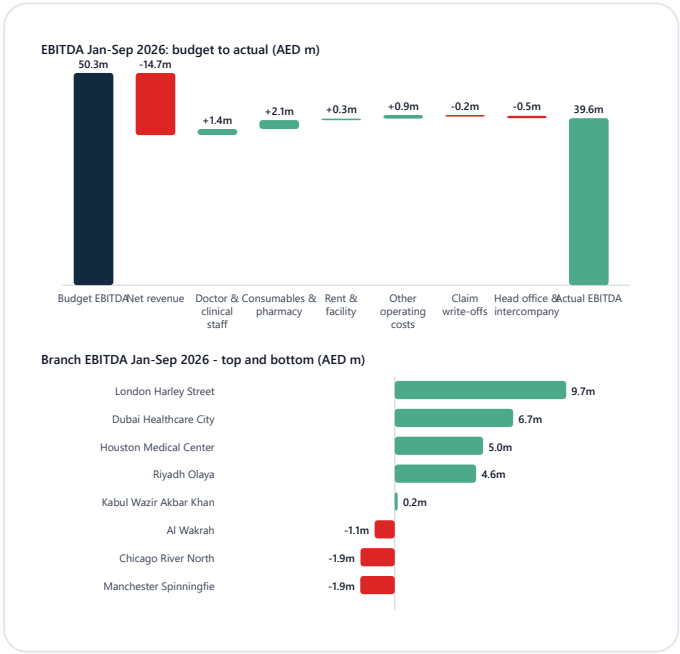
1. Renegotiate or re-code Najd Care claims (KSA). Rejections doubled in nine months; each percentage point of rejections removed is worth about AED 1.7m of EBITDA a year.

2. Put Chicago River North and Al Wakrah on a 90-day plan. Both are loss-making: Chicago lost a dermatologist and patients; Al Wakrah runs at roughly half its capacity.

3. Report the UK and Pakistan in constant currency. Their operating growth is strong; the AED view understates it. Hedge or price in local inflation.

4. Manchester is on a ramp, not a failure. Track utilisation monthly against the ramp curve before any decision.

5. Doctor capacity. Utilisation averages 77.9%; 28 doctors are below 65% - reschedule before hiring.



DATA INTEGRITY

Numbers you can defend

The Data Checks page recomputes thirteen independent totals and shows a publish gate. All pass:

Check	Result
GL revenue = visits x tariff - discounts	difference 0
Balance sheet balances for every entity, every month	difference 0
Cash flow: opening + net flows + FX effect = closing cash	difference 0
Revenue - write-offs - receivables movement = cash	difference 0
Intercompany fees and investments eliminate	difference 0
Doctor visits = patient visits + tests	difference 0
Control totals: revenue, EBITDA, cash, translation reserve, budget	match the generator

DELIVERABLES

What is in the folder

File	Content
Ravaan Medical Group FP&A.pbip	Power BI project: 13 pages in the HAS FP&A pack layout, 464 measures, star schema, scenario tables
Ravaan - Consolidation & FX Translation.xlsx	Entity TBs, rates, translation formulas, eliminations, consolidated P&L / BS / CF, CTA roll-forward, constant currency
Data*.csv	15 tables: 14.7k GL rows, 18k visit rows, 4.2k doctor-months, claims, cash, receivables ageing, FX
Ravaan - Measure Dictionary.xlsx / Chart Guide.pdf	Every measure explained; every chart explained

Skills shown: FP&A variance analysis (budget, prior year, constant currency), IFRS translation and consolidation, revenue-cycle analytics, unit economics, scenario modelling, dimensional modelling, DAX, Power Query, Python data generation and reconciliation.

Ravaan Medical Group is fictional; all names, insurers, doctors and figures are generated for this case study. Built by Jamil, FP&A and Power BI. Data through September 2026.